

Psychiatry

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[Psychiatry > Grief > Kubler-Ross] Easy - VERIFIED

Q32. According to Kübler-Ross's stages of grief (DABDA), a profound feeling of "hopelessness" is characteristically observed during which stage?

- A) Denial
- B) Anger
- C) Bargaining
- D) Depression

Answer: D) Depression

Explanation:

Correct - D: DABDA = Denial, Anger, Bargaining, Depression, Acceptance. Depression = reality sets in → hopelessness, withdrawal, sadness. Patient quiet, cries.

Incorrect - A: Denial = "This can't be happening". Shock phase.

Incorrect - B: Anger = "Why me?" Rage at God, doctors, family.

Incorrect - C: Bargaining = "If I live, I'll..." False hope, deals with God.

Trick: DABDA sequence yaad rakho. Depression = 4th stage, Acceptance = final.

Source: NORCET-2023 | Townsend Psychiatry

Tags: #PYQ #HighYield #Psychiatry #Grief #DABDA #Conceptual

[Psychiatry > Substance Abuse > Stimulants] Medium - VERIFIED

Q4. A patient is admitted to the emergency room experiencing hallucinations, agitation, and has an irritated nasal septum. Her friend reports she ingested a substance of abuse an hour ago. Which substance is most likely responsible?

- A) Heroin
- B) Cocaine
- C) LSD
- D) Marijuana

Answer: B) Cocaine

Explanation:

Correct - B: Cocaine snorting = nasal septum perforation/irritation, epistaxis. S/S: Agitation, hallucinations, tachycardia, HTN, mydriasis, hyperthermia. Vasoconstrictor effect.

Incorrect - A: Heroin = CNS depressant. Pinpoint pupils, resp depression, euphoria. IV use, not nasal usually.

Incorrect - C: LSD = visual hallucinations but no nasal damage. Oral.

Incorrect - D: Marijuana = relaxation, conjunctival injection, ↑appetite. No nasal septum damage.

Trick: "Cocaine = Coke nose". Irritated nose + wired = Cocaine. Heroin = Nodding off.

Source: NORCET-9 2025 | Kaplan Psychiatry

Tags: #PYQ #HighYield #Psychiatry #SubstanceAbuse #Emergency #Toxicology

[Psychiatry > Dementia > Behavioral] High - VERIFIED

Q5. Which of the following is an appropriate nursing intervention for a patient with dementia who develops a catastrophic reaction?

- A) Employ a negative response to the behavior
- B) Use touch to communicate
- C) Eliminate or reduce the environmental stimulation
- D) Maintain close personal boundaries

Answer: C) Eliminate or reduce the environmental stimulation

Explanation:

Correct - C: Catastrophic reaction = overwhelming stimuli → anxiety, agitation, aggression. Priority:

↓noise, ↓lights, ↓people. Calm, quiet environment. Redirect, don't argue.

Incorrect - A: Negative response = worsens agitation.
Incorrect - B: Touch may be perceived as threat during reaction. Ask permission first.
Incorrect - D: Close boundaries = invasion of space = ↑agitation. Give space.
Trick: "Catastrophe = Calm environment". Reduce STIMULI. Speak low, slow, simple.
Source: NORCET-9 2025 | Townsend Psychiatry
Tags: #PYQ #HighYield #Psychiatry #Dementia #Behavioral #Priority #NCLEX

[Psychiatry > Thought Process > Abnormalities] Medium - VERIFIED

Q6. The nurse is performing a Mental Status Examination (MSE) and the patient is continuously saying, without any context, "I want to go home, home, home." What can this be interpreted as?

- A) Perseveration
- B) Verbigeration
- C) Echopraxia
- D) Echolalia

Answer: B) Verbigeration

Explanation:

Correct - B: Verbigeration = meaningless repetition of words/phrases. "Home home home". Seen in schizophrenia, catatonia.

Incorrect - A: Perseveration = persistent reply to diff questions with same answer. "Q: Name? A: Home. Q: Age? A: Home."

Incorrect - C: Echopraxia = copying movements.

Incorrect - D: Echolalia = copying speech of others.

Trick: "Verbi = Verb/word, geration = generate again". Persevere = stuck on topic. Echo = copy.

Source: NORCET-9 2025 | Kaplan Psychiatry

Tags: #PYQ #HighYield #Psychiatry #MSE #ThoughtProcess #Terminology

[Psychiatry > Cognitive > MMSE] Medium - VERIFIED

Q7. A patient's Mini-Mental State Examination (MMSE) score is 19. What does this score indicate?

- A) Normal
- B) Mild cognitive impairment
- C) Moderate cognitive impairment
- D) Severe cognitive impairment

Answer: B) Mild cognitive impairment

Explanation:

Correct - B: MMSE: 24-30 Normal, 19-23 Mild, 10-18 Moderate, <10 Severe. Score 19 = Mild CI.

Incorrect - A/C/D: 19 not normal, not moderate/severe.

Trick: "MMSE: 24+ Normal, 19-23 Mild, 10-18 Moderate, <10 Severe". Remember ranges.

Source: NORCET-9 2025 | Psychiatry Scales

Tags: #PYQ #HighYield #Psychiatry #MMSE #Cognitive #Assessment #Formula

[Psychiatry > Eating Disorders > Anorexia] High - VERIFIED

Q14. Nursing intervention for anorexia nervosa patient not gaining weight:

- A) Give IV fluids
- B) Observe patient during and after meals
- C) Provide high-calorie diet
- D) Give antidepressants

Answer: B) Observe patient during and after meals

Explanation:

Correct - B: Anorexia = hide food, purge, exercise after meals. Priority: Direct observation during meals + 1-2hr after to prevent purging/exercise. 1:1 supervision.

Incorrect - A: IV fluids not first line unless severe dehydration.

Incorrect - C: High-calorie given but patient won't eat if not observed.

Incorrect - D: Antidepressants adjuvant, not priority for weight.

Trick: "Anorexia = Watch them Eat". Trust issues. Contract + supervise.

Source: NORCET-2020 | Townsend Psychiatry

Tags: #PYQ #HighYield #Psychiatry #EatingDisorder #Anorexia #Priority #NCLEX

[Psychiatry > ECT > Types] Medium - VERIFIED

Q15. Difference between regular and modified ECT is:

- A) No anesthesia in modified
- B) Anesthesia is given in modified
- C) Current is not used in modified
- D) Muscle relaxants not used in modified

Answer: B) Anesthesia is given in modified

Explanation:

Correct - B: Modified ECT = GA + Muscle relaxant + O₂ + Bite block. Prevents fractures, memory loss less. Current same.

Incorrect - A: Regular/Direct ECT = no anesthesia, high risk fractures.

Incorrect - C/D: Current used in both. Muscle relaxant used in modified.

Trick: "Modified = Made safe with Anesthesia". Now only modified used.

Source: NORCET-2020 | Psychiatry

Tags: #PYQ #HighYield #Psychiatry #ECT #Procedure

[Psychiatry > Pharma > Antipsychotics] Medium - VERIFIED

Q20. Psychiatric drug not requiring test dose:

- A) Haloperidol
- B) Clozapine
- C) Chlorpromazine
- D) Imipramine

Answer: A) Haloperidol

Explanation:

Correct - A: Haloperidol = typical antipsychotic. No test dose needed. Start low dose. Risk: EPS, NMS.

Incorrect - B: Clozapine = test dose 12.5mg needed due to agranulocytosis, hypotension, seizure risk.

Incorrect - C: Chlorpromazine = test dose for hypotension.

Incorrect - D: Imipramine TCA = test dose for cardiac effects.

Trick: "Clozapine = Caution, test dose". Haloperidol safe to start.

Source: NORCET-2020 | Psychopharmacology

Tags: #PYQ #HighYield #Psychiatry #Pharma #Antipsychotics #Safety

[Psychiatry > Delirium > Management] High - VERIFIED

Q21. Priority in delirium management during the first 24–48 hours is:

- A) Administer sedatives
- B) Orientation to person, place, environment
- C) Provide restraints
- D) Increase fluid intake

Answer: B) Orientation to person, place, environment

Explanation:

Correct - B: Delirium = acute confusion. Priority: Reorient, calm environment, clocks, calendar, familiar objects. Treat cause. Avoid restraints.

Incorrect - A: Sedatives worsen delirium. Last resort Haloperidol low dose.

Incorrect - C: Restraints ↑agitation, injury. Avoid.

Incorrect - D: Fluids important but orientation first.

Trick: "Delirium = Disoriented = Reorient". Safety + orientation + treat cause.

Source: NORCET-2020 | Townsend Psychiatry

Tags: #PYQ #HighYield #Psychiatry #Delirium #Priority #NCLEX

[Psychiatry > Pharma > Mood Stabilizers] High - VERIFIED

Q42. Advice for a patient on lithium therapy is:

- A) Restrict fluids
- B) Drink plenty of water
- C) Avoid salt intake
- D) Take drug on empty stomach

Answer: B) Drink plenty of water

Explanation:

Correct - B: Lithium = narrow therapeutic index 0.6-1.2 mEq/L. Dehydration → ↑Li levels → toxicity.

Advise: 2.5-3L fluid/day, consistent salt intake, avoid NSAIDs/diuretics. S/S toxicity: Tremors, vomiting, diarrhea, ataxia.

Incorrect - A: Restrict fluids = toxicity.

Incorrect - C: Avoid salt = toxicity. Low Na → Li reabsorption ↑.

Incorrect - D: Take with food to ↓GI upset.

Trick: "Lithium = Lots of water, Level salt". Dehydration = Death.

Source: NORCET-2020 | Psychopharmacology

Tags: #PYQ #HighYield #Psychiatry #Pharma #Lithium #PatientEducation #Toxicity

[Psychiatry > Pharma > Antipsychotics] High - VERIFIED

Q44. Frequency of blood test for clozapine therapy is:

- A) Daily
- B) Weekly
- C) Monthly
- D) Yearly

Answer: B) Weekly

Explanation:

Correct - B: Clozapine = agranulocytosis risk. REMS program: CBC weekly x 6 months, then biweekly x 6 months, then monthly. ANC must be >1500. Stop if <1000.

Incorrect - A/C/D: Not daily/yearly. Weekly initially mandatory.

Trick: "Clozapine = Count WBC Weekly". Most dangerous side effect = agranulocytosis.

Source: NORCET-2020 | FDA Clozapine REMS

Tags: #PYQ #HighYield #Psychiatry #Pharma #Clozapine #Monitoring #Safety

[Psychiatry > Eating Disorders > Anorexia] High - VERIFIED

Q47. Clinical features of anorexia nervosa include:

- A) Tachycardia, hypertension, hyperthermia
- B) Bradycardia, hypotension, hypothermia
- C) Tachypnea, hyperthermia, hypertension
- D) None of the above

Answer: B) Bradycardia, hypotension, hypothermia

Explanation:

Correct - B: Anorexia = starvation → ↓metabolism → Bradycardia <60, Hypotension <90/60, Hypothermia <36°C. Also lanugo hair, amenorrhea, constipation. Refeeding syndrome risk.

Incorrect - A/C: Tachy/HTN/hyperthermia = opposite, seen in hyperthyroid.

Trick: "Anorexia = All Low". HR, BP, Temp, Weight, K+, Phosphate all low.

Source: NORCET-2020 | DSM-5 Psychiatry

Tags: #PYQ #HighYield #Psychiatry #EatingDisorder #Anorexia #Assessment #NCLEX
